TCH → trastuzumab continuation

Default: six 21-day TCH cycles with intravenous (IV) trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose subcutaneous (SC) trastuzumab alternatives; the team confirms the formulation and visit schedule.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart

Trastuzumab

Keep echo appointments; report breathlessness or swelling.

2 Hands / feet

Docetaxel

Report numbness or tingling before the next dose.

3 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

4 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

5 Fluid / skin / nails

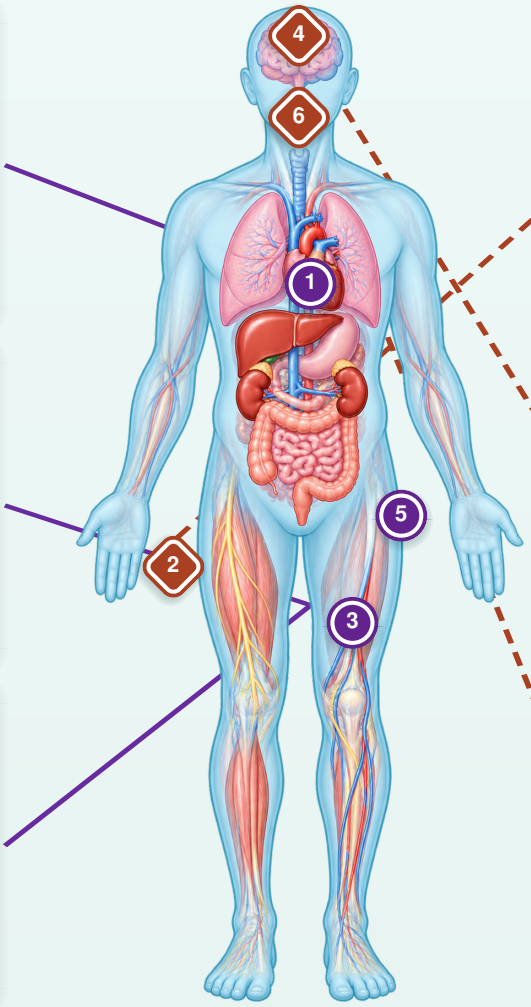
Docetaxel

Report swelling, painful skin or nail changes.

6 Mouth

Chemotherapy phase

Painful ulcers or difficulty drinking need advice.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Carboplatin

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Allergic reaction, sometimes after previous exposure
- Kidney or electrolyte changes

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

The correct supportive prescription

Follow the script for the active medicine, route and treatment day. Do not carry chemotherapy anti-sickness medicines automatically into an antibody-only phase.

Tests before treatment

NCCP specifies baseline and regular full blood count (FBC), liver and renal profiles; the team confirms the pre-dose checks for the active chemotherapy or trastuzumab-only phase. Verify kidney function for carboplatin dosing.

Keep heart checks on the plan

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

Plan for symptoms

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00258/

Sources and review status are available online. Follow your own prescription and hospital instructions.